



PROTOCOL

Community CAMHS Entry Protocol

Scope (Staff):	All Staff
Scope (Area):	CAMHS Community

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

**This protocol is currently under review to be incorporated into the relevant Model of Care.
The content remains current throughout this review process.**

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1. Aim

This protocol describes the entry process to Community Child and Adolescent Mental Health Services (Community CAMHS).

2. Risk

A child and family/carer may not gain entry to a service that meets their needs in a timely manner.

3. Background

Community CAMHS provide services for infants, children and young people (up to their 18th birthday). Ten (10) Community CAMHS clinics are located across the Perth metropolitan area, though appointments may also be provided at the child's home or in their school or another appropriate agreed venue (e.g. Youth centre, doctors office) based on clinical need and as agreed with partner agencies. Community CAMHS operates during business hours from Monday to Friday, with out of hours emergencies directed to [CAMHS Crisis Connect](#). All Community CAMHS sites comply with disability access requirements.

Community CAMHS offers evidence-based individual, family and group interventions. Delivery of care is underpinned by the Choice and Partnership Approach (CAPA), which is a clinical system that focusses on the child and their family being at the centre of the care planning process. Subject to availability, children and their families are given a choice of when they would like to attend an appointment. Community CAMHS aims to ensure that all referrals that are considered to be suitable for the service are seen at a Choice appointment with 28 days of referral. In the CAPA model the clinician's role is viewed as a "facilitator with expertise", and the service aims to allocate each client to a clinician with skills and experience relevant to their individual needs. Community CAMHS work is multidisciplinary, involving clinicians from a variety of professions.

Community CAMHS aims to engage children and families in all aspects of service culture, functioning and delivery. Current examples of child and family' involvement in Community CAMHS include;

- Collection of consumer and carer feedback on services;
- Active involvement of consumers in their own health care, and
- Collaboration with consumer reference groups

To facilitate access for children and their families, Community CAMHS aims to maintain effective and collaborative partnerships with a range of services and organisations. This includes, but is not limited to, local mental health services, Emergency Departments (EDs), Non-Government Organisations, General Practitioners (GP) and Alcohol and Other Drug services.

4. Definitions

Access: Ability of consumers or potential consumers to obtain required or available services when needed and within an appropriate time.

Child: infant, child, adolescent or young person under the age of 18 (i.e. until their 18th birthday)

Choice appointment: The initial face-to-face (or telehealth) contact between the family and one or more of the service Clinicians.

Choice co-ordinator/clinician: Select members of the CAMHS team who have the necessary expertise and skills to ensure that children and their families are directed to the most appropriate care pathway.

Entry: the process provided by the mental health service which assists the consumer and their carers to make contact with the mental health service and receive appropriate assistance.

Parent: any parent, legal guardian, responsible family member, or other identified carer of the child.

Partnership: Process whereby families meet with a mental health clinician to participate in assessment and treatment planning, involving a range of shared treatments and interventions provided by the clinical team.

Triage: Tasks performed by a clinician post receipt of a referral, where the information in the referral is not sufficient to be able to make a decision as to whether a Choice appointment should be offered. The primary aim of Triage is to obtain sufficient information to be able to determine whether a Choice appointment should be offered or not, and whether the Choice appointment should be Urgent or Routine.

5. Key points

- The '[Charter of Mental Health Care Principles](#)' informs CAMHS community services, including Principle 3, Person-centred approach; Principle 4, Delivery of treatment care and support; Principle 5, Choice and self-determination, Principle 6, Welcoming all from diverse background, and Principle 7, Care that respects Aboriginal culture and spirituality.
- Recovery oriented practice informs our services. It:
 - Provides a holistic framework that informs all contact with children, and their families;
 - Builds and enhances strength, resilience, and social and emotional wellbeing;
 - Supports children to return to normal developmental pathways;
 - Is underpinned by the premise that children do recover from mental health problems;
 - Engages all areas of the child, and family's life, including relationships, education, vocation, and leisure; and

- Informs any recovery plan that is regularly reviewed by the child or, family and multi-disciplinary team.
- Community CAMHS gives consideration to the cultural and linguistic needs of the child and family at all stages of the entry process:
 - Where a referral is for a child who identifies as Aboriginal, the family must be offered access to an Aboriginal Mental Health Worker (where one is available) at the earliest possible opportunity. Services must refer to the CAMHS Aboriginal Mental Health Service Guideline (in development) for guidance in relation to the role of the Aboriginal Mental Health Worker throughout the child and family's engagement with Community CAMHS;
 - When a family's cultural background is unknown at the time of referral to Community CAMHS, the Choice coordinator / clinician must seek this information from the family prior to the Choice appointment, in order to ensure that a culturally sensitive practice can be offered;
 - Prior to all Choice appointments, the Choice coordinator / clinician must identify whether an interpreter is required and organise this service as appropriate; and
 - Where practicable, consideration is given to the gender, culture, ethnicity and diversity characteristics of the Choice clinician.

6. Referral Criteria

As a specialist mental health service, Community CAMHS will prioritise seeing children and young people who are experiencing severe, complex and persistent mental health symptoms that impact on psychosocial functioning (at home, school or socially), and/or who present with significant risk due to their mental health difficulties.

The reason for entry to CAMHS must primarily relate to severe and complex mental health problems, although other concurrent and/or associated difficulties may exist (e.g. autism, intellectual disability, and child protection issues).

Refer to [Appendix 1](#) for details of the referral pathway to Community Psychiatry from Acute CAMHS services.

The range of presenting problems usually accepted on referral includes (but is not limited to):

- Psychotic symptoms;
- Severe and complex mood related symptoms;
- Severe and complex anxiety symptoms (e.g. severe generalised social anxiety, obsessive compulsive disorder, panic disorders, phobias);
- Persisting suicidal ideation, recent suicide attempt, and/or serious risk of harm to self or others due to severe and complex mental health problems;
- Eating disorders;

- Co-morbid severe and complex mental health symptoms related to substance use; and
- Severe and complex symptoms resulting from trauma.

All referrals are screened by the Choice coordinator / clinician, in consultation with the Consultant Psychiatrist, to determine the appropriate response from the service. Referrals are prioritised on the basis of clinical need and risk in relation to mental health concerns.

7. Referral Management

- Referrals are accepted via email, letter or facsimile. Use of the [Community CAMHS Referral Form](#) is encouraged.
- All referrals are screened by the Choice Coordinator within one working day of receipt of referral. Referrals received outside business hours are screened the following business day.
- All referrals are responded to by the Choice Coordinator as soon as practicable.
- Referrals are usually accepted for children from within designated catchment areas (see [Community CAMHS Locations and Catchment Postcodes](#)). Referrals from outside the catchment area may be accepted if clinically appropriate and where this is considered to be the best outcome for the child and family. Decision-making regarding out-of-catchment referrals must also take into account transport options and services' capacity to undertake home and community visits.

Refer to [Appendix 2](#) for details of the Referral to Choice pathway including triage.

7.1 Where an alternative service is best placed to meet the needs of the child / family

- The Choice coordinator / clinician will consider redirecting referrals where:
 - the child is over 18 years of age;
 - the request is not primarily for assessment and treatment of mental health symptoms;
 - the request is solely for the purposes of medication review or Psychiatry review;
 - the request primarily relates to assessment for family court, other legal purposes or for providing letters of support to other agencies;
 - In this instance, the child can be seen for their mental health symptoms but not for the sole purpose of the preparation of a report.
 - See the [CAHS Legal Requests for Care and Treatment/ Expert Opinion Reports policy](#)

- CAHS Mediation and Legal Support Services should be contacted for advice on requests for reports, see [CAHS Legal Services intranet site](#).
- the request primarily relates to diagnostic assessment for neurodevelopmental disorders (e.g. Attention Deficit Hyperactivity Disorder, Autism Spectrum Disorder, Fetal Alcohol Spectrum Disorder).
 - Community CAMHS recommends referral to a Developmental Paediatrician for children who are the subject of concerns regarding neurodevelopmental disorders.
 - Paediatricians may refer patients to Community CAMHS where there are severe co-morbid mental disorders.
- **Note:** A CAMHS Consultant Psychiatrist must have oversight of all decisions to redirect a child and family to another service. If the Choice coordinator / clinician forms a view that an alternative service is best placed to meet the needs of the child and family, they must consult with a CAMHS Consultant Psychiatrist. The discussion with a Consultant Psychiatrist and their approval of the decision to redirect the child and family must be documented in the child's medical record and on PSOLIS.
- The referral outcome, and suggestions on appropriate alternative services (if available), must be communicated to the referrer. A copy of this communication must be sent to the family.

7.2 Referral meets criteria - Routine referral process

- The child and family are invited to book into a Choice Appointment by letter or telephone. They are offered a Choice appointment at a venue and at a time that is clinically indicated and suitable for both the service and the family.
- Where the family agrees to access the service for a Choice appointment, the family contacts the service to book the appointment. Administrative staff must forward a confirmation letter to the child and family without delay.

7.3 Referral meets criteria - Urgent referral process

- When an accepted referral is deemed urgent, the Choice coordinator / clinician must ensure that attempts are made to contact the child or family via telephone within two business days. The Choice coordinator / clinician will attempt to undertake crisis / safety planning with the child and family, and identify services that the child and family may wish to contact while waiting for their Choice appointment.
- Consideration should be given to whether ACRT would be appropriate for urgent referrals that are not able to be actioned by Community CAMHS.
- Where the service is unable to contact the child or family within this timeframe, a letter is sent inviting the family to contact the service to book into a Choice Appointment. The Choice coordinator / clinician must make all reasonable efforts to inform the referrer and professionals involved in the care of the child that

telephone contact was unable to be made. All efforts to contact the family, referrer, and other professionals involved in the care of the child should be documented in the child's medical record and on PSOLIS. Refer to the [CAMHS Clinical Engagement policy](#) for further guidance.

- An urgent appointment will be offered based on clinical need.

7.4 Referral does not contain sufficient information - Triage

- In this scenario, the Choice coordinator / clinician must make contact with the referrer, child, family, and/or other services to obtain further information about the referral. The triage process concludes when the Choice coordinator / clinician has obtained sufficient clinical information to make a decision as to whether or not a Choice appointment should be offered, and whether the Choice appointment should be Urgent or Routine.
- Throughout the triage process, clinical consultation and support is available from the Service Manager and/or Consultant Psychiatrist.
- For referrals for children with complex needs (e.g. children in care), the triage process may include a case conference with the referrer and other service providers in order to clarify the roles and expectations of CAMHS, if this does not delay urgent care.
- If, at the conclusion of the triage process, it is deemed that the child's needs are best met through another service, refer to [section 7.1](#).

7.5 Referral outcomes

- All referral outcomes are communicated to the referrer, including whether or not CAMHS is considered to be the most appropriate service for the child and family. A copy of this communication must be sent to the family.

8. Choice Appointment

- All appropriate referrals to Community CAMHS must be offered an initial face-to-face meeting (Choice appointment), or via tele-health if required.
 - **Referrals from PCH CAMHS, ACRT and Specialised CAMHS:** If there has been sufficient assessment by the referring service, a collaborative meeting involving the referrer, child, family and the receiving Community CAMHS team may take the place of a Choice appointment. The child and family may proceed straight to Partnership if this is the agreed outcome. The aim of the Choice appointment is to undertake a preliminary assessment and reach a shared understanding of both the child's mental health symptoms, and the child and family's needs and expectations.
 - ACRT will receive referrals from CAMHS Crisis Connect and Community CAMHS
- Risk assessment in Choice is essential, and should be conducted in line with the [CAMHS Risk Assessment and Management policy](#).

- The clinician/s must work with the child and family to explore which services that are best placed to help. This may be the Community CAMHS clinic, an alternate service, or multiple agencies.
- A range of options can then be discussed, and the clinician/s will help the family to make an informed choice about what to do next.

8.1 If it is agreed that the child / family will proceed to Partnership

- Families who agree to Partnership will be offered further appointments for assessment and recovery planning. See the [CAMHS Clinical Assessment policy](#).
- Families must be made aware that they are not considered to be 'active' clients until after they have attended their Partnership appointment. While waiting for their Partnership appointment, families should be encouraged to implement any risk / safety planning measures identified at Choice, and to contact [CAMHS Crisis Connect](#) or their local ED with any urgent mental health concerns.
- All families who come into Community CAMHS for Partnership must be reviewed at the multidisciplinary team meeting, as per the [CAMHS Multidisciplinary Team Clinical Reviews policy](#).

8.2 If an alternative service is best placed to meet the needs of the child / family

- Community CAMHS will aim to identify appropriate alternative services and provide the family with contact information for those services, in a sensitive and supportive manner. Consideration should be given to whether ACRT would be appropriate for urgent referrals that are not able to be actioned by Community CAMHS.
- **Note:** A CAMHS Consultant Psychiatrist must have oversight of all decisions to redirect a child and family to another service. If a Consultant Psychiatrist is not present at the time of the Choice appointment, the relevant clinician/s must consult with them as soon as practicable after the decision is made. The discussion with a Consultant Psychiatrist and their approval of the decision to redirect the child and family must be documented in the child's medical record and on PSOLIS.

9. Unplanned presentations / "Walk-Ins"

9.1 For children who are active with your team:

See actions outlined in the [CAMHS Clinical Engagement policy](#).

9.2 For children who are not active with your team:

The administrative staff member must contact the Choice coordinator / clinician or another senior clinician. If neither is available, the administrative staff member must contact the Service Manager or Head of Service. The contacted clinical staff member must then provide appropriate advice based on the information provided by the child and family at the time. This may include (but is not limited to):

- Advice to self-present to the closest emergency department;
- Advice to consult their GP; or

- The offer to book into a Choice appointment.

10. Re-entry

- If a child and family require re-entry following discharge from a Community CAMHS clinic, all efforts must be made to provide the child with a choice between seeing the same Partnership clinician or a different Partnership clinician. The ability of the service to meet the child's request will be subject to their clinical presentation at the time of re-entry, and the caseload and workload of the requested Partnership clinician.
- Each referral must be judged on its own merits against the referral criteria regarding the child's current context and presenting concerns.
- Re-referrals for children with complex needs may require a case conference with the referrer and other service providers in order to clarify roles and expectations of CAMHS.

Related CAHS internal policies, procedures and guidelines
CAHS Legal Requests for Care and Treatment / Expert Opinion Reports
CAMHS Clinical Assessment policy
CAMHS Clinical Engagement policy
CAMHS Multidisciplinary Team Clinical Reviews policy
CAMHS Patient Rights Under The Mental Health Act
CAMHS Risk Assessment and Management policy

References and related external legislation, policies, and guidelines
Smoke Free WA Policy
York A, and Kingsbury, S. The Choice and Partnership Approach: A Guide to CAPA. Surrey; 2009.
Australian Government. National Standards for Mental Health Services. Canberra: Commonwealth of Australia; 2010.
Mental Health Foundation- National CAMHS Support Service. Evaluation of the Choice and Partnership Approach in Child and Adolescent Mental Health Services in England. 2009.

Robotham D, James K, Cyhlarova E. Managing demand and capacity within child and adolescent mental health services: an evaluation of the Choice and Partnership Approach. The Mental Health Review. 2010;15(3):22.

Useful resources (including related forms)

[CAHS Mediation and Legal Support Services](#)

[Charter of Mental Health Care Principles](#)

[Community CAMHS Referral Form](#)

[Community CAMHS Catchment Areas](#)

This document can be made available in alternative formats on request.

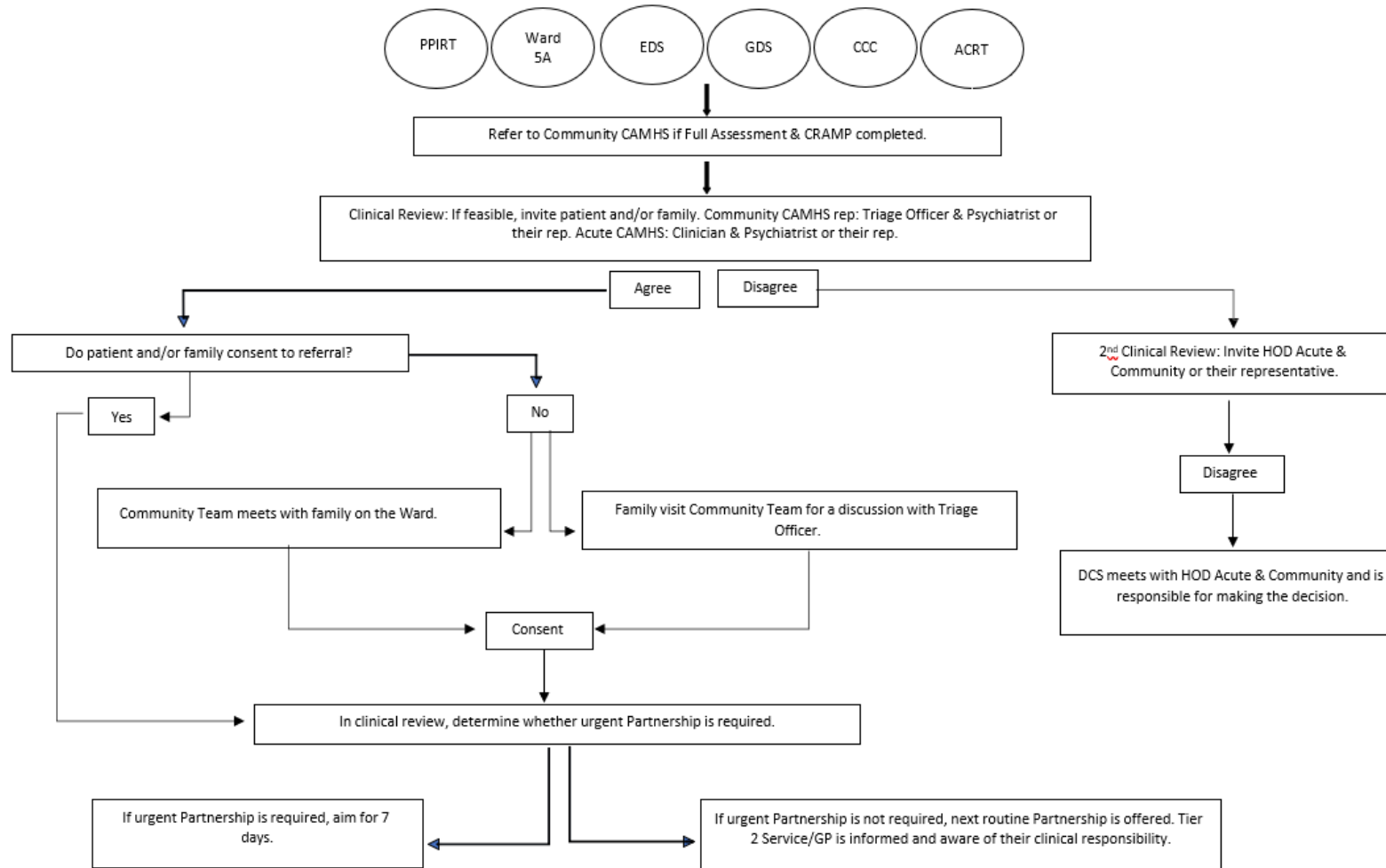
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*Note: CAMHS Director Clinical Services approved an extension of the review date to 09/08/2026.

Appendix 1: Referral to Community Psychiatry from Acute CAMHS



Note: if family do not consent to a referral to Community CAMHS once they have received the necessary information about community CAMHS and understand the risks of not engaging with community CAMHS then the Acute Service in conjunction with Community service needs to decide whether it is appropriate to consider whether a Community Treatment Order (CTO) under MHA is indicated. If it is not indicated, then the Acute Service needs to explore alternative options for follow up with the family. Acute Services can seek information on tier 2 services from triage officer in relevant Community Clinic

Appendix 2: Referral to Choice pathway including triage

